

SECOND AMENDED STATEMENT OF FACTS AND CONTENTIONS (FORM 9A)

WC/2024/227 – Cory Lea Shepherd (Appellant) v Workers' Compensation Regulator (Respondent). Section 550(4), Workers' Compensation and Rehabilitation Act 2003.

Particulars are given by reference to the paragraph numbers of the Appellant's notice to admit facts served on the Respondent on 28 August 2026, to which the Respondent responded on 8 September 2026.

PART A

Appellant: Cory Lea Shepherd. Employer: State of Queensland (Queensland Health) – Logan Hospital Switchboard. Role: Administration Officer, Switchboard Services, classification AO3, a continuous shift working role.

Injury: major depressive disorder with anxious distress (DSM-5 296.23), diagnosed by the Appellant's treating psychiatrist on 24 October 2024.

The injury is a diagnosed medical condition arising from the Appellant's employment over the course of conduct pleaded at Part B.2 below, between approximately June 2023 and June 2024. It is not pleaded as arising from an event occurring on a single day. The date of injury recorded on the work capacity certificates of the treating general practitioner is 18 June 2024. That date is the practitioner's and is a matter for the medical evidence; the Appellant asserts no date of onset of his own.

Particulars: paragraphs 1 to 4, 13 to 16 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART B — MATERIAL FACTS

The documentary facts identified below have been admitted to the extent recorded in the Respondent's response of 8 September 2026. Those admissions provide evidence from which the Commission is invited to determine the nature, reasonableness and causal significance of the relevant circumstances.

1. Baseline — the medical record

1.1 The Switchboard receives emergency response notifications and distributes them to the response groups by category of emergency, and maintains the contact and paging information by which that is done.

Duty engaged: paragraphs 5, 7 to 8, 10 of that notice (the role description).

1.2 **Prior health.** Before 18 June 2024 the Appellant had not been diagnosed with or treated for depression. The general-practice record of 16 November 2023 (Dr Nanayakkara) records poor sleep with shift work, that the Appellant could not do shifts without a good sleep, "no psychological illness such as depression or psychosis", and mood good, with melatonin and temazepam prescribed that day. **[Tab M1 — general-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024, obtained by the Respondent under the Form 29 signed 4 July 2025; Respondent's item 11.]** Relied upon as the contemporaneous record that shift work was affecting the Appellant's sleep, and of what was prescribed for it, before any claim or proceeding. Its clinical significance is a matter for the treating doctors. Private entries unrelated to the injury are redacted on the extracted pages and marked as such.

Particulars: paragraphs 2, 13, 214, 218, 263 to 265, 269 to 271 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.3 **The referral of 16 May 2024.** The referral letter of 16 May 2024 (Dr Zhao) renews a referral to a psychiatrist, Dr Amini, for "ongoing care and management". It lists the past medical history as the history-list items "26/10/2022 ADHD" and "26/10/2022 Anxiety", and lists the medications then current, **which include no antidepressant, no anxiolytic and no other psychotropic medication.** **[Tab M1.]** It was written the day after the email of 15 May 2024 at 6:23 pm asking the Appellant to retract his email of that afternoon. That sequence is pleaded as to date only; no connection between the two is alleged. No appointment with the psychiatrist was obtained on that referral.

Particulars: paragraphs 76 to 77 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.4 **Deterioration and first presentation.** The Appellant last worked a shift on 3 June 2024. On days in May 2024 and in the first week of June 2024 he drove to work, was unable to enter the building, and reported himself unfit from the car park. The date of injury recorded on the certificates is 18 June 2024. On 28 June 2024 (Dr Slawinski) the record shows "stress at work" and "upset by people not following rules", reason for visit anxiety. On 1 July 2024 (Dr Hawes) the record shows "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works later than earlier", and "causing anxiety". **[Tab M1.]** The events those records name are the matters admitted on 8 September 2026 and pleaded at Part B.2 below. The records are relied upon as the contemporaneous record of what was reported, before any claim decision, dismissal or proceeding; not as a finding of fact as to mechanism.

1.5 **Certification.** The work capacity certificates of Dr Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024, state the date of injury as 18 June 2024, that the Appellant was first seen for this injury on 1 July 2024, and certify no functional capacity continuously from 1 July to 6 October 2024. The referral to a psychiatrist is recorded on the certificate of 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates". **[Tab M2; Respondent's items 7 and 8.]** The medication box is unticked on each certificate.

1.6 **Diagnosis, 24 October 2024.** At the first consultation, on the referral, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". The Appellant's email of 5:12 pm that day reads "I have finally been able to see a psychiatrist today". **[Tab M3; Respondent's item 9.]** Read with 1.5, it records the progression from the injury certified as anxiety and stress on 1 July 2024 to a depressive disorder by 24 October 2024.

1.7 **The report of 13 February 2025.** Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). Severity, functional effect and treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician records the origin as "workplace stress stemming from issues with management and rostering"; that the issues "began approximately one year ago when a new manager was appointed"; that pay was "withheld or delayed for up to five months at a time, leading to significant financial stress"; and that "premature exposure to the workplace is more likely result in significant deterioration". **[Tab M4; Respondent's item 10.]** The stressors so recorded correspond to matters admitted on 8 September 2026: management and rostering, and the night-shift line; the shorter break; and pay withheld or delayed from the 5 February 2024 fortnight, uncorrected at

13 May, "claims older than 3 months" on 28 May, a claim effective 30 March recorded "Part Completed" on 30 May, and the two February claims absent from the myHR report.

Particulars: paragraphs 193, 196 to 197, 203, 210 to 212, 220, 258 to 260 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.8 The report's footer, and the author's position. The report bears a footer reading "for the only reason of clinical information and not for medico-legal use". The author's emails of 5 and 8 September 2026 state of his records "You can use them according to the need to support your legal issues", and identify the report of 13 February 2025 as "the report that captures the relevant information you have requested" on the matters in issue. **[Tab M5.]** The report is not relied upon for attribution among the individual events pleaded at Part B.2, nor for any matter after 24 October 2024 as a cause of the injury.

1.9 Current capacity. The Employee Capability Checklist completed by Dr Day Hong Ma on 3 July 2026 records current capacity and restrictions and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors". **[Tab M6.]** Relied upon for effect and capacity only, not for causation; it post-dates 1 July 2024.

1.10 Outstanding, and the finding in the decision under appeal. The clinical records of the treating psychiatrist from 24 October 2024 were offered by the practice on 5 September 2026 and have been requested; they are not yet received and will be served on receipt. On the medical evidence then before it, the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a psychological nature" and stated: "Having regard to the medical evidence, I am satisfied your employment was a **significant contributing factor** to the psychological injury". The claim was rejected under section 32(5), not section 32(1). **[Tabs M7 and M9.]** The hearing is de novo and that finding does not bind the Commission; it is relied upon as an admitted document.

Particulars: paragraphs 261 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

2. Causative stressors (composite course, June 2023 – June 2024)

Stressor 1 — the conditions in which the work was carried on

(a) The directory, and the removal of the means of correcting it. On 18 July 2023 access to the database was removed from all operators, the Contact and Number Changes book was taken out of the room, and an after-hours correction was to wait until one of two named persons returned. The Respondent does not allege access was restored before 18 June 2024. The duty to keep the information accurate remained on the position throughout. From 15 April 2024 the same two persons were also the after-hours contact, and from 14 May 2024 the same person was the route for notifying unavailability.

Duty engaged: paragraphs 6 to 7 of that notice (the role description).

Particulars: paragraphs 39 to 55, 111 to 113, 162 to 166, 273 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(a1) The criticality of the function, as admitted by the Respondent. On 18 February 2026 the Respondent admitted that "Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety", adding that there was a procedure in place for this to occur, and denied a further paragraph on the ground that there was already a procedure in place. The procedure in place from 18 July 2023 is the procedure pleaded at (a) above: the operator could not amend an entry, and an amendment was to be sought from one of two persons, one of whom worked one and a half days a fortnight in a temporary project role. Its operation between 2 and 20 May 2024 is pleaded at (h) below.

Particulars: paragraphs 283, 289 to 291 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(b) The Communication Book entry of 6 June 2023. A reminder concerning the monthly update of medical contact numbers was removed. The Respondent admits the removal in its pleading and again on 8 September 2026, does not allege the page has been located or copied, and does not list the Communication Book. On 18 February 2026 the Respondent admitted that Ms Taylor removed a page from the workplace communications book, and did not admit that the page contained the Appellant's handwriting "because the respondent does not have a copy of the page". It also admitted that in an email to Ms Reese of 6 June 2023 Ms Taylor stated: "I did raise my voice and asked him to please stop talking over the top of me."

Duty engaged: paragraphs 9 of that notice (the role description).

Particulars: paragraphs 143 to 153, 288, 292 of the notice to admit facts served 28 August 2026, admitted 8 September 2026. Paragraph 154 of that notice is not admitted.

(c) The matters raised in August and September 2023, and the response.

Particulars: paragraphs 26 to 38, 155 to 161 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(d) The special pandemic leave application, February 2024. Submitted three times, declined twice, approved on the same material.

Particulars: paragraphs 114 to 142 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(e) The complaint of 13 May 2024. The Appellant made a complaint to the Metro South Health Ethical Standards Unit, which that Unit determined on 24 December 2024 constituted a public interest disclosure, reference 24-ESU-1130. **The content is not set out, and no connection between it and any subsequent conduct is pleaded or relied upon in this appeal.**

(f) The office hours, the retraction request, and the answer. On 15 May 2024 the Appellant asked that directives and changes be made in consultation and asked for the manager's office hours. He was asked that evening to retract that email. Two days later the hours were sent to the whole department. On 21 May he was asked which particular changes or directives had concerned him. Between 23 August 2023 and 17 May 2024 no fixed hours had been stated to staff. The Appellant pleads no characterisation of any person: the stated hours are pleaded because they determine how long an incorrect directory entry remained uncorrected.

Duty engaged: paragraphs 3 to 4 of that notice (the role description).

Particulars: paragraphs 70 to 88 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(g) Workplace representation. The Appellant notified an interest in the Switchboard delegate role; the Union Encouragement Policy QH-POL-248 requires managers to take a positive, supportive role; he was endorsed as a workplace delegate on or about 3 November 2025. **No allegation of deliberate suppression is made.**

Particulars: paragraphs 178 to 181, 293 to 294 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(h) What arrived at the console, and the obligation to consult. On 3 and 8 May 2024 the MASPER Registrar reported nine occasions of calls reaching the wrong medical team; the first reply came five days, eighteen hours and fourteen minutes later and sought the reporting clinician's hours; nothing is alleged to have been sent to the Switchboard before 10:15 am on 9 May. On 15 and 20 May the Integrated Respiratory Service asked twice for a directory amendment and reported it could not assist patients; the Appellant escalated the cause within three hours and two minutes; no document of the discussion, no communication to the Service, no amendment and no notification to staff is listed. Clause 3.2 of the Queensland Public Health Sector Certified Agreement (No. 11) 2022 requires contribution to decision-making "not only in appearance, but in fact" and "an actual and genuine opportunity to

influence the outcome, before a final decision is made"; the change of 15 April 2024 was effective the day it was notified.

Duty engaged: paragraphs 8, 11 to 12 of that notice (the role description).

Particulars: paragraphs 56 to 69, 89 to 110, 167 to 181 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(i) What the employer states about its own systems.

Particulars: paragraphs 263 to 272 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

Matters pleaded as context only

The matters at (c), (d), (e) and (g) above, and at paragraph 3 below, are pleaded as part of the sequence of matters the Appellant raised and the responses he received. They are not relied upon as independent causative circumstances, and no retaliatory or reprisal connection between any of them and any other conduct is alleged. The causative circumstances relied upon are those at (a), (b), (f), (h) and (i) above and at Stressors 2 and 3 below.

Stressor 2 — remuneration

(a) Between February and May 2024 the Appellant's pay required correction. On 3 May 2024 Payroll identified four fortnights and asked the manager to submit an Attendance Variation and Allowance Claim for each; on 13 May Payroll directed the Appellant to his line manager; on 21 May the manager was awaiting payroll confirmation; on 28 May the Appellant was asked to sign a validation of claims older than three months. The myHR submissions report records the manager as the initiator of each of the five such claims in the period and the Appellant as the initiator of none.

(a1) **Public holidays on which the Appellant was not required to work.** Section 23 of the Hospital and Health Service General Employees (Queensland Health) Award provides that where an employee in receipt of the additional week's leave prescribed by clause 19.2(a) is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, that employee is to be paid an additional day's wage or granted a day's holiday in lieu. The Appellant was rostered off on public holidays between February and April 2024. As at 1 May 2024 payroll was still reviewing his entitlements regarding public holidays not required, while the manager confirmed that since his commencement of full-time employment all payments for public holidays not required had been processed.

Particulars: paragraphs 241, 255 to 256 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(a2) **Rostering practices.** On 20 May 2024 the Director wrote to LBH_HR following up a query raised in relation to an email received from a staff member about the department's rostering practices, attaching the Queensland Health Fatigue Risk Management Systems Implementation Guideline.

Particulars: paragraphs 222 to 223 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

Duty engaged: paragraphs 2, 13 of that notice (the role description).

Particulars: paragraphs 182 to 206 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(b) The Respondent pleads that any discrepancies were remedied in a timely manner and that there are no outstanding underpayments. The claim submitted on 28 May 2024 is recorded as "Part Completed", and neither claim reference identified by Payroll on 3 May 2024 appears in the report.

Particulars: paragraphs 203 to 204, 207 to 210, 296 to 298 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

Stressor 3 — the roster and fatigue

(a) Rostering errors on the Appellant's line were acknowledged in writing by the Director. On 10 May 2024 she wrote to Human Resources that the Appellant's roster would not be considered and that he was not yet aware of it. On 21 May she asked him to identify the changes and directives about which he had concerns.

Duty engaged: paragraphs 2, 13 of that notice (the role description).

Particulars: paragraphs 211 to 223 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(a1) **The rostering history.** A rostering error in August 2023 was acknowledged by the manager, who offered to roster the Appellant off the following day "to give you the required rest period", and by the Director, who accepted there had been "a rostering error that was accidentally" made. The Appellant applied in writing to increase his hours to a full-time rotational roster, confirmed he was able and willing to work any roster presented to him including the full 24-hour rotational schedule, expressed willingness to take on additional night shifts, and provided a draft roster spreadsheet. In November and December 2024 the employer released a Consultation Paper and outcome proposing a more equitable roster with redistribution of nights based on FTE and a rotational roster ensuring fair distribution of penalties.

Particulars: paragraphs 28, 30 to 31, 33, 35, 156 to 157, 159, 167 to 181, 287 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(a2) **The Respondent's pleaded position on the roster.** On 18 February 2026 the Respondent denied paragraphs of the Appellant's earlier notice on the grounds "because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable". In November 2024 the employer released a Consultation Paper proposing a roster "intended to introduce a more equitable roster for Switchboard Services", including "Redistribution of nights for greater equity based on FTE" and a "Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties".

Particulars: paragraphs 171 to 173, 299 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(b) The shifts of 17 and 18 March 2024 were separated by a seven-hour break. The Award requires a break of not less than ten hours, and eight hours applies instead of ten only in specific circumstances. The 8-hour agreement of 17 June 2020 applies only where staff-initiated shift swaps have occurred, and the Respondent does not allege these shifts arose from such a swap.

Particulars: paragraphs 17 to 25, 224 to 227, 232 to 234, 257, 284 to 286 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(b1) **The standard, as admitted.** On 18 February 2026 the Respondent admitted that "The Employer's 'Fatigue Risk Management Policy' and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement", and that the shifts of 17 and 18 March 2024 resulted in "a break of only 7 hours". It denied a further paragraph on the ground of the agreement of June 2020. The Respondent pleads that the Appellant could refuse shifts at any time; the role description states, under mandatory requirements, that the position is a continuous shift working role and that the occupant must be able to work a roster covering multiple shifts over a 24/7 period.

Duty engaged: paragraphs 13 of that notice (the role description).

(c) The emergency workload carried on those shifts is recorded in the employer's Emergency Code Register. Those paragraphs were not admitted by the Respondent and will be proved by the register and any other admissible evidence.

Particulars: paragraphs 228 to 231 of the notice to admit facts served 28 August 2026.

(d) On 8 April 2024 the Appellant requested a review of payment and raised fatigue. The enquiry was escalated to Human Resources on 9 April. No response is alleged between 9 April and 1 May, an interval of 23 days, when the payment was refused. The leave of 19 March 2024 came from the Appellant's own leave.

Particulars: paragraphs 235 to 237, 239 to 240, 242 to 256 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

(d1) **The leave of 19 March 2024, and the employer's own policy.** The Leave Takings Report produced by Metro South Hospital and Health Service records the leave taken on 19 March 2024 as Leave Category "Sick", Leave Type "Sick Leave", Time Code "SCK". The Respondent pleads that the leave was paid, and that clause 18.10 of the Award denies fatigue leave because no overtime was performed. The employer's own fatigue policy of June 2020, as recorded in the review decision, states that many problems regarding fatigue leave can be overcome by not rostering an employee who is on call for the first shift of the following day, and by not rostering on call an officer who is rostered for the first shift on the following day.

(e) The Respondent's own review decision of 24 October 2024 finds that the break equated to seven hours, that the rostering of the two shifts amounted to unreasonable management action, that the Appellant sustained a personal injury of a psychological nature, and that his injury arose out of employment where employment was a significant contributing factor.

Particulars: paragraphs 257 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

3. Subsequent conduct

(a) Meetings scheduled while the Appellant held current medical certificates. (b) Employment ended under abandonment provisions in October 2024 while certificates were current, and the Appellant was reinstated to his employment with an effective date of 20 September 2024, the reinstatement taking effect despite any payroll documentation. (c) The documents the Appellant provided to WorkCover Queensland in July and August 2024, and what the Respondent's amended List of Documents records of them.

Particulars: paragraphs 274 to 282 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART C — CONTENTIONS

1. **■ The onus, accepted.** The Appellant accepts that he bears the onus of proving every element of his case on the balance of probabilities, including that his injury did not arise out of, or in the course of, reasonable management action taken in a reasonable way (Workers' Compensation Appeal Guide, Part 7.3; *Prizeman v Q-COMP* [2005] ICQ 43, where Hall P held that the onus is on the worker to establish that management action was unreasonable or taken in an unreasonable way, and that in the absence of evidence to support that assertion management action must be considered reasonable). The Appellant does not contend that the Respondent bears that onus, and the contrary contention in the Amended Form 9A filed 7 April 2026 is withdrawn.

2. **How the onus is discharged.** *Prizeman* presumes reasonableness only in the absence of evidence. The evidence is now on the record and is not in dispute. In particular the Respondent admits, as to its own documents and its own pleading: that access to the database was removed and is not alleged to have been restored before 18 June 2024; that no document recording consultation with Switchboard operators before the change of 15 April 2024 is listed; that nine occasions of misdirected calls were reported by a clinician and nothing is alleged to have been sent to the Switchboard about them for six days; that no document of the discussion of 20 May 2024, no communication to the Integrated Respiratory Service, and no amendment to the contact-details document is listed; that the fatigue records do not exist, that the training applied only to health practitioners and clinical assistants, that fatigue assessment at the Switchboard began only after 30 June 2024, and that there were no consequential changes to operating procedures; and that complaints were managed solely by email or verbally with the complainant.

3. **Compensable injury — section 32(1).** The Appellant sustained a personal injury of a psychological nature arising out of, or in the course of, his employment, employment being a **significant contributing factor**. The injury is the diagnosed condition identified at Part A, arising from the course of conduct pleaded at Part B.2 and not from an event on a single day. The Respondent's own review decision so finds.

4. **Section 32(5)(a), first limb — the matters at Stressor 1(a) and 1(h) are not management action.** A directory the occupant of the position is required to keep accurate but cannot correct; emergency notifications reaching the wrong team; a clinical service unable to assist patients because a directory entry is wrong — these are the conditions in which the work was carried on, not action taken by the employer against the worker. One decision was taken, on 18 July 2023, for a stated project reason; what followed is a state of the system, and subsection (5) does not reach it.

5. **Section 32(5)(a), second limb — where the matters pleaded are management action, it was not taken in a reasonable way.** The change of 15 April 2024 was effective the day it was notified, with no record of prior consultation, against an instrument requiring a genuine opportunity to influence the outcome before a final decision. The remuneration correction was returned across four fortnights to the only person able to make it and is recorded as Part Completed. The fatigue enquiry drew no response for 23 days.

6. **Delaney.** Where multiple management actions form a course of conduct, their reasonableness may be evaluated in context and as a whole. That evaluation must nevertheless first identify the circumstances that constitute management action. Operational conditions that are not management action do not become management action merely because they occurred alongside management decisions.

7. **Mahaffey.** The presence of reasonable management action among the circumstances contributing to an injury does not necessarily enliven the exclusion where an unreasonable management action, or a circumstance that is not management action, also had a sufficient causal connection to the disorder. The rostering of the shifts on 17 and 18 March 2024 was found in the review decision to constitute unreasonable management action. The Appellant relies upon the medical and contemporaneous evidence to establish its contribution as part of the pleaded course of causative circumstances.

8. **The effect of the admissions.** The admissions are made for this proceeding under rule 49 and are relied upon as evidence of the existence and terms of the documents admitted, and of the matters the Respondent's statement of facts and contentions and List of Documents do not contain. The Appellant does not contend that any admission determines the reasonableness of any conduct; that is a matter for the Commission.

9. **The hearing de novo.** The Respondent says no finding in Review Decision 69983 binds the Commission. That is accepted, and it applies to the whole of the decision. The conclusion under appeal is a conclusion of that decision, reached upon the findings recorded in it.

10. **The Respondent's pleaded case.** Paragraph 27 of the amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. No disciplinary process, formal performance management or communication described as a warning before 18 June 2024 is alleged.

Particulars: paragraphs 238, 295, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART D — ORDERS SOUGHT

1. The appeal be allowed. 2. The decision of the Respondent dated 24 October 2024 be set aside. 3. It be declared that the Appellant sustained an injury within the meaning of section 32 of the Workers' Compensation and Rehabilitation Act 2003. 4. The Appellant's application for compensation be accepted. 5. Costs reserved.